



# Glucoflow

Diabetes progression, ready before the visit

Reports arrive separately. Staff reconstruct the history,  
then the doctor searches for earlier results during the visit.

*Built and running locally: records are read, fields are proposed with their source,  
and staff approve every fact before it reaches the progression view.*

# A clinic morning with Glucoflow

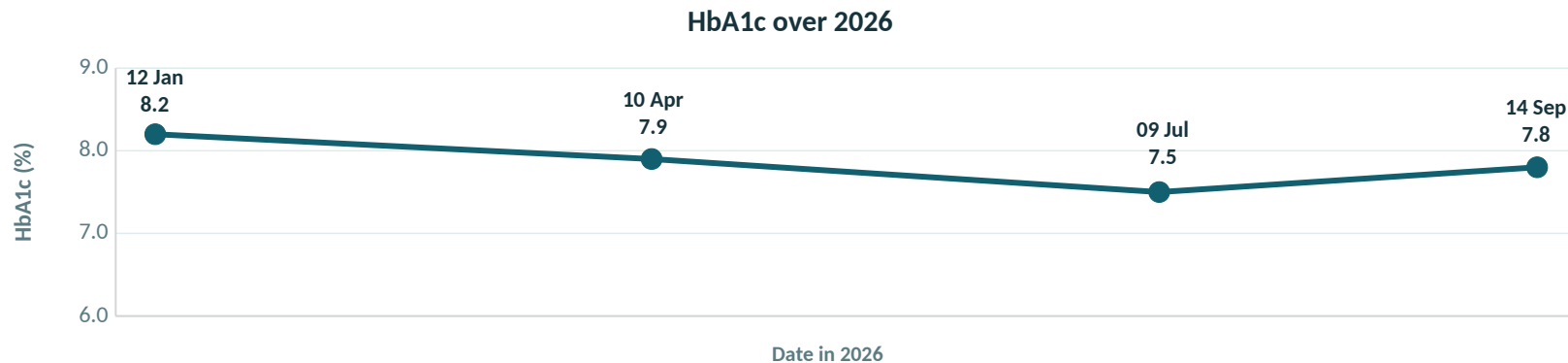
**Proposed return visit: one new report joins the clinic's existing patient file**

| When                | Owner and action                                                           | Output                              |
|---------------------|----------------------------------------------------------------------------|-------------------------------------|
| At check-in         | Clinic assistant uploads one new report during the usual check-in.         | Draft history                       |
| Before consultation | A trained intake reviewer checks the new facts while the patient waits.    | Approved update                     |
| During consultation | Doctor opens the approved progression view from the existing patient file. | Dated history and source references |

**If review cannot finish before the visit, the doctor uses the original reports. The draft stays on hold until staff approve it.**

# Progression in context

Three dated HbA1c results • verified local build • synthetic patient P0482



Other dated source records, listed chronologically

15 Jan • Prescription

Metformin listed

18 Apr • Prescription

Prescription revised

09 Jul • Patient note

Diet and walking reported

04 Sep • Eye exam

Report available

HbA1c

4 results • 7.5–8.2%

Glucose

5.6 mmol/L • 14 Sep

LDL cholesterol

2.1 mmol/L • 14 Sep

BP / weight

no record — gap

Kidney (eGFR/UACR)

no record — gap

Eye / foot / nerve

eye exam 04 Sep

Each value opens its source report; each care event retains its date and source.  
The doctor interprets the history. Timing alone does not establish a treatment effect.

Dated and source-linked; absent records stay visible as gaps. Assay methods stay with the source; no cross-lab correction.

# One fact, checked at its source

Implemented extraction and staff review. Synthetic report; deterministic fixture engine, not a live model.

OCR reads the report. AI maps varied layouts and prescription wording into dated fields with source evidence.

Sutra

P0482 | Review new record

CONCEPT MOCKUP

Source report · page 1

SYNTHETIC LAB REPORT

Patient: P0482  
Collected: 14 Sep 2026

| Test              | Result | Unit   |
|-------------------|--------|--------|
| HbA1c             | 7.8    | %      |
| Fasting glucose   | 5.6    | mmol/L |
| Total cholesterol | 4.1    | mmol/L |
| HDL cholesterol   | 1.3    | mmol/L |
| LDL cholesterol   | 2.1    | mmol/L |
| Triglycerides     | 1.0    | mmol/L |

Report D · page 1

Check the extracted fact

Test

HbA1c

Date

14 Sep 2026

Value

7.8

Draft unit: mmol/L — needs correction

Corrected unit

%

[Open source](#)

Approve correction

Awaiting staff approval

The example catches a wrong unit. Staff approve the correction before the fact joins the history.

Built: Vite / React, Capacitor Android, Fastify API + worker, PostgreSQL. Running locally; hosted deployment is configured but not yet exercised.

# The return visit: review the change

Verified on web and on an Android emulator: approved facts stay available while the new report is reviewed

Sutra

P0482 | Return visit

CONCEPT MOCKUP

## Approved history — reused

3 HbA1c results · 4 care events

|             |      |            |
|-------------|------|------------|
| 12 Jan 2026 | 8.2% | ✓ Approved |
| 10 Apr 2026 | 7.9% | ✓ Approved |
| 09 Jul 2026 | 7.5% | ✓ Approved |

Previous sources and review versions retained

## One new report

Report D · 14 Sep 2026

HbA1c

7.8%

✓ Unit correction approved

✓ 6 measurements checked against source

✓ Patient identity confirmed

Release updated packet

Approved history + new verified facts

SYNTHETIC DATA

FOR WORKFLOW DESIGN ONLY

First intake checks all imported facts. A return visit checks the new report and amendments. Previous approvals stay available.

# The application workflow, as built

Architecture as built. Staff approve facts before they enter the visit view.

01

## Read records

Upload PDFs or scans. Read digital text directly and use OCR for image pages.

02

## Propose fields

AI maps varied layouts and wording to dates, values and units with source evidence.

03

## Check and approve

Validate patient, date and unit. Staff correct the draft against its source.

04

## Save and deliver

Save approved versions. Explore trends, search reports and export the visit summary.

Changed source records reopen affected facts. An unresolved patient identity conflict keeps the update on hold.

## Later extension: consented ABHA records

Verify the clinic patient link and request scoped consent. Retrieve permitted provider records through ABDM, validate their structured format and send them through the same review.

Expiry or revocation stops access. Enforce the consent's erasure deadline. Sandbox and certification precede production.

Implemented locally. NHA ABDM M1 / M3 documentation; NRCeS FHIR R4. No live integration claimed.

# Does the return visit save work?

Illustrative workload model. Every minute below is an assumption to test; extraction accuracy is measured, time is not.

## 10 min

Manual timeline

## 6 min

Glucoflow return visit

| Task                     | Manual (min) | Glucoflow (min) |
|--------------------------|--------------|-----------------|
| Intake                   | 2            | 1               |
| Verification and sources | 5            | 3               |
| Doctor lookup            | 2            | 1               |
| Corrections              | 1            | 1               |
| Total                    | 10           | 6               |

## The comparison test

30 unseen synthetic histories. Use the same reports in both workflows. Test first and return visits separately.

Use separate participants and balanced task order. Sum time across all roles, including corrections and failed extraction.

Independent reference: patient, test, date, value and unit. Score precision and recall. Report identity, date and unit errors separately.

**Gate: at least 30% less median return time, with no worse precision or recall. Identity conflicts block release.**

**Illustrative clinic day: 20 return visits × 4 minutes = 80 minutes saved across staff and doctor roles.**

First intake: Glucoflow 12 min vs manual 10 min. One return visit offsets the assumed 2-minute setup penalty.

Evaluation designed and extraction accuracy measured on synthetic corpora. No time savings measured yet. Reviewer time is included. Existing extraction and trend tools: Eka Care Smart Report

# The first build and clinic pilot

Team details to confirm: practising doctor, technical lead and responsibilities. Clinic pilot access remains pending.



## Clinic use

Patients upload and add notes.  
Staff review source facts.  
Doctors explore dated history.  
Buyer: clinic owner or operations lead.  
Test pricing in the pilot.



## First build

Web and Android  
Patient and clinic accounts.  
Upload, review and approve.  
Explore progression and notes.  
Search reports and open sources.  
Save history and export.  
Built and verified locally.



## Demo and pilot

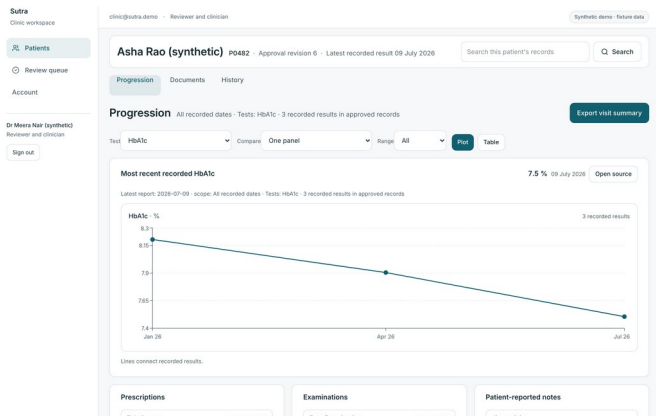
Synthetic demo  
Patient uploads on Android.  
Clinic reviews on web or phone.  
Explore progression, find a report and export the saved history.  
Then measure time and accuracy.

Differentiation to test: reuse approvals and review only new facts. Compare this workload with report extraction and trend tools.

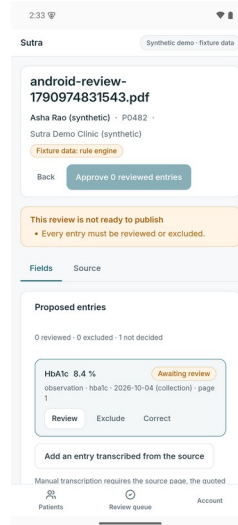
**Next: run the live model, deploy, and time the return visit in a 60–90 day clinic pilot**

# Verified locally, on web and Android

Screenshots from the running build: synthetic patient P0482, deterministic fixture extraction, Android emulator.



Clinic web: approved progression with its coverage scope



Android: reviewer checks one entry



Android: patient records

Verified: upload → processing → review → approval → progression on both roles; 95 automated tests, 25 end-to-end API checks and 43 browser checks pass.

Not yet verified: live model extraction (fixture rule engine only), hosted authentication and storage, a physical Android device, and any time saving.